Naranjo Scoring Worksheet and Causality Justification

Case	Drug and event	Dechallenge	Rechallenge	Score	Category
SYN-PV-011	Metformin and nausea	Unknown	Positive	5	Probable
SYN-PV-012	Amlodipine and peripheral oedema	Unknown	Positive	5	Probable
SYN-PV-013	Ciprofloxacin and tendon pain	Negative	Positive	5	Probable
SYN-PV-014	Sertraline and dizziness	Negative	Negative	2	Possible
SYN-PV-015	Omeprazole and headache	Unknown	Positive	5	Probable
SYN-PV-016	Losartan and fatigue	Positive	Negative	2	Possible
SYN-PV-017	Amoxicillin and rash	Unknown	Positive	5	Probable
SYN-PV-018	Atorvastatin and myalgia	Unknown	Positive	5	Probable
SYN-PV-019	Metformin and nausea	Positive	Negative	3	Possible
SYN-PV-020	Amlodipine and peripheral oedema	Unknown	Not done	3	Possible
SYN-PV-021	Ciprofloxacin and tendon pain	Positive	Not done	4	Possible
SYN-PV-022	Sertraline and dizziness	Negative	Positive	5	Probable
SYN-PV-023	Omeprazole and headache	Negative	Negative	2	Possible
SYN-PV-024	Losartan and fatigue	Positive	Positive	5	Probable
SYN-PV-025	Amoxicillin and rash	Negative	Positive	5	Probable
SYN-PV-026	Atorvastatin and myalgia	Unknown	Positive	5	Probable
SYN-PV-027	Metformin and nausea	Unknown	Positive	5	Probable
SYN-PV-028	Amlodipine and peripheral oedema	Unknown	Negative	2	Possible
SYN-PV-029	Ciprofloxacin and tendon pain	Unknown	Positive	5	Probable
SYN-PV-030	Sertraline and dizziness	Positive	Positive	6	Probable

Totals: 13 Probable, 7 Possible, 0 Definite, 0 Unlikely.

Case SYN-PV-011: Metformin and nausea

23 years, sex unknown | Metformin 5 mg once daily, oral | Indication: Hypertension | Onset: 19 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Recovering | Reporter: Consumer

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 19 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovering, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because nausea started 19 days after Metformin was started. Stomach effects are usually seen near the start or after a dose increase, so 19 days is a little late but still possible.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovering, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Nausea is a known reaction of Metformin (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Metformin (500 to 3000 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Hypertension' does not match the usual use of Metformin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-012: Amlodipine and peripheral oedema

24 years, female | Amlodipine 5 mg once daily, oral | Indication: Hypertension | Onset: 4 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Not recovered (at follow-up) | Reporter: Pharmacist

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 4 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is not recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because peripheral oedema started 4 days after Amlodipine was started. Swelling from amlodipine usually appears within days to weeks, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is not recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Peripheral oedema is a known reaction of Amlodipine (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-013: Ciprofloxacin and tendon pain

75 years, female | Ciprofloxacin 500 mg twice daily, oral | Indication: Diabetes | Onset: 5 days
Dechallenge: Negative | Rechallenge: Positive | Outcome: Not recovered (at follow-up) | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 5 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	No	0	Dechallenge is recorded as negative (event did not improve after stopping).
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because tendon pain started 5 days after Ciprofloxacin was started. Tendon problems with ciprofloxacin can start within days to a few months, so this timing fits.

Dechallenge and rechallenge. Dechallenge is negative, so the event did not improve after stopping (0). Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is negative, so these two fields should be checked together.

Other evidence and alternative causes. Tendon pain is a known reaction of Ciprofloxacin (Q1, +1). Vitamin D is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (diabetes) can by itself cause tendon pain, so this is a possible alternative explanation that should be checked. Age 75 is a recognised risk factor for tendon problems with ciprofloxacin, which supports a drug effect but does not change the score.

Data problems I noticed. The indication 'Diabetes' does not match the usual use of Ciprofloxacin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want details of the restart, the medical history and any objective test results.

Case SYN-PV-014: Sertraline and dizziness

75 years, female | Sertraline 5 mg once daily, oral | Indication: Diabetes | Onset: 9 days
Dechallenge: Negative | Rechallenge: Negative | Outcome: Recovering | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 9 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	No	0	Dechallenge is recorded as negative (event did not improve after stopping).
4	Reappeared on re-administration?	No	-1	Rechallenge is recorded as negative (event did not come back).
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+2**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because dizziness started 9 days after Sertraline was started. Dizziness with sertraline is often seen early in treatment but can occur later, so this timing is acceptable.

Dechallenge and rechallenge. Dechallenge is negative, so the event did not improve after stopping (0). Rechallenge is negative (-1). The event did not come back, which goes against the drug as the cause. A negative dechallenge does not fit well with the outcome of recovering.

Other evidence and alternative causes. Dizziness is a known reaction of Sertraline (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (diabetes) can by itself cause dizziness, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Sertraline (25 to 200 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Diabetes' does not match the usual use of Sertraline and should be confirmed.

Final classification. Total Naranjo score is 2, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 1, which would still be Possible. To firm this up I would want the medical history and any objective test results.

Case SYN-PV-015: Omeprazole and headache

33 years, male | Omeprazole 20 mg once daily, oral | Indication: Hypertension | Onset: 16 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Recovered | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 16 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because headache started 16 days after Omeprazole was started. Headache with omeprazole is often seen early in treatment but can occur later, so this timing is acceptable.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Headache is a known reaction of Omeprazole (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (hypertension) can by itself cause headache, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The indication 'Hypertension' does not match the usual use of Omeprazole and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-016: Losartan and fatigue

66 years, sex unknown | Losartan 20 mg once daily, oral | Indication: Diabetes | Onset: 10 days
Dechallenge: Positive | Rechallenge: Negative | Outcome: Not recovered (at follow-up) | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	No	0	Reference field says Unexpected, so the pack shows no previous conclusive report.
2	Event after drug administered?	Yes	+2	Onset is recorded 10 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Yes	+1	Dechallenge is recorded as positive (event improved after stopping).
4	Reappeared on re-administration?	No	-1	Rechallenge is recorded as negative (event did not come back).
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+2**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because fatigue started 10 days after Losartan was started. Fatigue with losartan can appear within the first days to weeks, so this timing is reasonable.

Dechallenge and rechallenge. Dechallenge is positive, so the event improved after the drug was stopped (+1). Rechallenge is negative (-1). The event did not come back, which goes against the drug as the cause. A positive dechallenge does not fit well with the patient not being recovered at follow-up.

Other evidence and alternative causes. Fatigue is recorded as unexpected for Losartan, so Q1 scores 0. I would check this against the reference safety information, because fatigue appears in many losartan labels. No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (diabetes) can by itself cause fatigue, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (20 mg once daily) is outside the usual adult range for Losartan (25 to 100 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Diabetes' is vague for Losartan and should be confirmed.

Final classification. Total Naranjo score is 2, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 1, which would still be Possible. To firm this up I would want the medical history and any objective test results.

Case SYN-PV-017: Amoxicillin and rash

57 years, sex unknown | Amoxicillin 5 mg once daily, oral | Indication: Hypertension | Onset: 21 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Recovering | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 21 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovering, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because rash started 21 days after Amoxicillin was started. That is late for a delayed amoxicillin rash, which usually shows within about two weeks, so the timing is possible but not ideal.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovering, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Rash is a known reaction of Amoxicillin (Q1, +1). Vitamin D is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Amoxicillin (about 750 to 3000 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Hypertension' does not match the usual use of Amoxicillin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-018: Atorvastatin and myalgia

56 years, female | Atorvastatin 500 mg twice daily, oral | Indication: Anxiety | Onset: 17 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Not recovered (at follow-up) | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 17 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is not recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because myalgia started 17 days after Atorvastatin was started. Muscle pain with atorvastatin usually appears within weeks to months, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is not recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Myalgia is a known reaction of Atorvastatin (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (anxiety) can by itself cause myalgia, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (500 mg twice daily) is outside the usual adult range for Atorvastatin (10 to 80 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Anxiety' does not match the usual use of Atorvastatin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-019: Metformin and nausea

29 years, sex unknown | Metformin 5 mg once daily, oral | Indication: Anxiety | Onset: 9 days
Dechallenge: Positive | Rechallenge: Negative | Outcome: Recovering | Reporter: Pharmacist

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 9 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Yes	+1	Dechallenge is recorded as positive (event improved after stopping).
4	Reappeared on re-administration?	No	-1	Rechallenge is recorded as negative (event did not come back).
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+3**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because nausea started 9 days after Metformin was started. That is a reasonable time for a stomach effect of metformin.

Dechallenge and rechallenge. Dechallenge is positive, so the event improved after the drug was stopped (+1). Rechallenge is negative (-1). The event did not come back, which goes against the drug as the cause.

Other evidence and alternative causes. Nausea is a known reaction of Metformin (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (anxiety) can by itself cause nausea, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Metformin (500 to 3000 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Anxiety' does not match the usual use of Metformin and should be confirmed.

Final classification. Total Naranjo score is 3, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 2, which would still be Possible. To firm this up I would want the medical history and any objective test results.

Case SYN-PV-020: Amlodipine and peripheral oedema

56 years, sex unknown | Amlodipine 5 mg once daily, oral | Indication: Diabetes | Onset: 7 days
Dechallenge: Unknown | Rechallenge: Not done | Outcome: Recovering | Reporter: Pharmacist

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 7 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovering, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Unknown	0	Rechallenge is recorded as not done, so it scores as do not know or not done.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+3**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because peripheral oedema started 7 days after Amlodipine was started. Swelling from amlodipine usually appears within days to weeks, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovering, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge was not done (0).

Other evidence and alternative causes. Peripheral oedema is a known reaction of Amlodipine (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The indication 'Diabetes' does not match the usual use of Amlodipine and should be confirmed.

Final classification. Total Naranjo score is 3, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 2, which would still be Possible. To firm this up I would want the date the drug was stopped and when the event settled, the medical history and any objective test results.

Case SYN-PV-021: Ciprofloxacin and tendon pain

33 years, sex unknown | Ciprofloxacin 5 mg once daily, oral | Indication: Infection | Onset: 9 days
Dechallenge: Positive | Rechallenge: Not done | Outcome: Recovered | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 9 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Yes	+1	Dechallenge is recorded as positive (event improved after stopping).
4	Reappeared on re-administration?	Unknown	0	Rechallenge is recorded as not done, so it scores as do not know or not done.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+4**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because tendon pain started 9 days after Ciprofloxacin was started. Tendon problems with ciprofloxacin can start within days to a few months, so this timing fits.

Dechallenge and rechallenge. Dechallenge is positive, so the event improved after the drug was stopped (+1). Rechallenge was not done (0).

Other evidence and alternative causes. Tendon pain is a known reaction of Ciprofloxacin (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Ciprofloxacin (500 to 1500 mg/day), so it may be a unit or entry error, or a medication error if it is correct.

Final classification. Total Naranjo score is 4, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 3, which would still be Possible. To firm this up I would want the medical history and any objective test results.

Case SYN-PV-022: Sertraline and dizziness

43 years, female | Sertraline 5 mg once daily, oral | Indication: Infection | Onset: 21 days
Dechallenge: Negative | Rechallenge: Positive | Outcome: Not recovered (at follow-up) | Reporter: Consumer

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 21 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	No	0	Dechallenge is recorded as negative (event did not improve after stopping).
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because dizziness started 21 days after Sertraline was started. Dizziness with sertraline is often seen early in treatment but can occur later, so this timing is acceptable.

Dechallenge and rechallenge. Dechallenge is negative, so the event did not improve after stopping (0). Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is negative, so these two fields should be checked together.

Other evidence and alternative causes. Dizziness is a known reaction of Sertraline (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Sertraline (25 to 200 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Infection' does not match the usual use of Sertraline and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want details of the restart, the medical history and any objective test results.

Case SYN-PV-023: Omeprazole and headache

26 years, male | Omeprazole 500 mg twice daily, oral | Indication: Anxiety | Onset: 4 days
Dechallenge: Negative | Rechallenge: Negative | Outcome: Recovering | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 4 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	No	0	Dechallenge is recorded as negative (event did not improve after stopping).
4	Reappeared on re-administration?	No	-1	Rechallenge is recorded as negative (event did not come back).
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+2**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because headache started 4 days after Omeprazole was started. Headache with omeprazole is often seen early in treatment but can occur later, so this timing is acceptable.

Dechallenge and rechallenge. Dechallenge is negative, so the event did not improve after stopping (0). Rechallenge is negative (-1). The event did not come back, which goes against the drug as the cause. A negative dechallenge does not fit well with the outcome of recovering.

Other evidence and alternative causes. Headache is a known reaction of Omeprazole (Q1, +1). No concomitant medicines are reported. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (anxiety) can by itself cause headache, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (500 mg twice daily) is outside the usual adult range for Omeprazole (20 to 40 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Anxiety' does not match the usual use of Omeprazole and should be confirmed.

Final classification. Total Naranjo score is 2, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 1, which would still be Possible. To firm this up I would want the medical history and any objective test results.

Case SYN-PV-024: Losartan and fatigue

79 years, sex unknown | Losartan 500 mg twice daily, oral | Indication: Hypertension | Onset: 3 days
Dechallenge: Positive | Rechallenge: Positive | Outcome: Recovering | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	No	0	Reference field says Unexpected, so the pack shows no previous conclusive report.
2	Event after drug administered?	Yes	+2	Onset is recorded 3 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Yes	+1	Dechallenge is recorded as positive (event improved after stopping).
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because fatigue started 3 days after Losartan was started. Fatigue with losartan can appear within the first days to weeks, so this timing is reasonable.

Dechallenge and rechallenge. Dechallenge is positive, so the event improved after the drug was stopped (+1). Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed.

Other evidence and alternative causes. Fatigue is recorded as unexpected for Losartan, so Q1 scores 0. I would check this against the reference safety information, because fatigue appears in many losartan labels. Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (500 mg twice daily) is outside the usual adult range for Losartan (25 to 100 mg/day), so it may be a unit or entry error, or a medication error if it is correct.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want details of the restart, the medical history and any objective test results.

Case SYN-PV-025: Amoxicillin and rash

61 years, sex unknown | Amoxicillin 500 mg twice daily, oral | Indication: Infection | Onset: 21 days
Dechallenge: Negative | Rechallenge: Positive | Outcome: Recovered | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 21 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	No	0	Dechallenge is recorded as negative (event did not improve after stopping).
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because rash started 21 days after Amoxicillin was started. That is late for a delayed amoxicillin rash, which usually shows within about two weeks, so the timing is possible but not ideal.

Dechallenge and rechallenge. Dechallenge is negative, so the event did not improve after stopping (0). Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A negative dechallenge does not fit well with the outcome of recovered. A rechallenge only makes sense if the event settled first, but dechallenge is negative, so these two fields should be checked together.

Other evidence and alternative causes. Rash is a known reaction of Amoxicillin (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (infection) can by itself cause rash, so this is a possible alternative explanation that should be checked.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want details of the restart, the medical history and any objective test results.

Case SYN-PV-026: Atorvastatin and myalgia

48 years, sex unknown | Atorvastatin 20 mg once daily, oral | Indication: Hypertension | Onset: 12 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Not recovered (at follow-up) | Reporter: Physician

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 12 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is not recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because myalgia started 12 days after Atorvastatin was started. Muscle pain with atorvastatin usually appears within weeks to months, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is not recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Myalgia is a known reaction of Atorvastatin (Q1, +1). Vitamin D is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The indication 'Hypertension' does not match the usual use of Atorvastatin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-027: Metformin and nausea

24 years, sex unknown | Metformin 20 mg once daily, oral | Indication: Hypertension | Onset: 14 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Recovered | Reporter: Pharmacist

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 14 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because nausea started 14 days after Metformin was started. That is a reasonable time for a stomach effect of metformin.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Nausea is a known reaction of Metformin (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Data problems I noticed. The reported dose (20 mg once daily) is outside the usual adult range for Metformin (500 to 3000 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Hypertension' does not match the usual use of Metformin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-028: Amlodipine and peripheral oedema

77 years, sex unknown | Amlodipine 5 mg once daily, oral | Indication: Hypertension | Onset: 18 days
Dechallenge: Unknown | Rechallenge: Negative | Outcome: Recovered | Reporter: Pharmacist

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 18 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	No	-1	Rechallenge is recorded as negative (event did not come back).
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+2**	**Causality category: Possible (1 to 4)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because peripheral oedema started 18 days after Amlodipine was started. Swelling from amlodipine usually appears within days to weeks, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is negative (-1). The event did not come back, which goes against the drug as the cause.

Other evidence and alternative causes. Peripheral oedema is a known reaction of Amlodipine (Q1, +1). Vitamin D is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0.

Final classification. Total Naranjo score is 2, which is Possible (1 to 4). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 1, which would still be Possible. To firm this up I would want the date the drug was stopped and when the event settled, the medical history and any objective test results.

Case SYN-PV-029: Ciprofloxacin and tendon pain

35 years, sex unknown | Ciprofloxacin 5 mg once daily, oral | Indication: Diabetes | Onset: 13 days
Dechallenge: Unknown | Rechallenge: Positive | Outcome: Recovered | Reporter: Consumer

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 13 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Unknown	0	Dechallenge is unknown. The patient is recovered, but the report does not say when this happened compared with stopping the drug.
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+5**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because tendon pain started 13 days after Ciprofloxacin was started. Tendon problems with ciprofloxacin can start within days to a few months, so this timing fits.

Dechallenge and rechallenge. Dechallenge is unknown (0). The patient is recovered, but the report does not say when recovery happened compared with stopping the drug, so I did not count it as a positive dechallenge. Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed. A rechallenge only makes sense if the event settled first, but dechallenge is unknown, so these two fields should be checked together.

Other evidence and alternative causes. Tendon pain is a known reaction of Ciprofloxacin (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (diabetes) can by itself cause tendon pain, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (5 mg once daily) is outside the usual adult range for Ciprofloxacin (500 to 1500 mg/day), so it may be a unit or entry error, or a medication error if it is correct. The indication 'Diabetes' does not match the usual use of Ciprofloxacin and should be confirmed.

Final classification. Total Naranjo score is 5, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 4, which would change the category to Possible. To firm this up I would want the date the drug was stopped and when the event settled, details of the restart, the medical history and any objective test results.

Case SYN-PV-030: Sertraline and dizziness

82 years, male | Sertraline 20 mg once daily, oral | Indication: Anxiety | Onset: 17 days
Dechallenge: Positive | Rechallenge: Positive | Outcome: Recovering | Reporter: Consumer

Naranjo scoring worksheet

Q	Question	Answer	Score	Basis in the record
1	Previous conclusive reports?	Yes	+1	Reaction is listed as Known in the reference field of the pack.
2	Event after drug administered?	Yes	+2	Onset is recorded 17 day(s) after treatment began.
3	Improved on withdrawal/antagonist?	Yes	+1	Dechallenge is recorded as positive (event improved after stopping).
4	Reappeared on re-administration?	Yes	+2	Rechallenge is recorded as positive (event came back). The narrative does not describe the restart.
5	Alternative causes?	Unknown	0	No medical history is given, so alternative causes can be neither confirmed nor excluded.
6	Reappeared with placebo?	Unknown	0	No placebo information in the report.
7	Drug level in toxic range?	Unknown	0	No drug level reported.
8	Dose-response relationship?	Unknown	0	No dose change described.
9	Similar previous reaction?	Unknown	0	No previous reaction to this or a similar drug reported.
10	Objective evidence?	Unknown	0	No lab test or other objective confirmation reported.
Total score			**+6**	**Causality category: Probable (5 to 8)**

Causality justification

Time relationship. I scored Q2 as Yes (+2) because dizziness started 17 days after Sertraline was started. Dizziness with sertraline is often seen early in treatment but can occur later, so this timing is acceptable.

Dechallenge and rechallenge. Dechallenge is positive, so the event improved after the drug was stopped (+1). Rechallenge is positive, so the event is recorded as coming back when the drug was restarted (+2). This is the strongest evidence in the case, but the narrative does not describe the restart, so it needs to be confirmed.

Other evidence and alternative causes. Dizziness is a known reaction of Sertraline (Q1, +1). Paracetamol is also listed. I do not think it is the likely cause, but a small contribution cannot be fully ruled out. The pack gives no medical history, placebo information, drug level, dose change, previous reaction or objective test, so Q5 to Q10 are all unknown and score 0. The listed indication (anxiety) can by itself cause dizziness, so this is a possible alternative explanation that should be checked.

Data problems I noticed. The reported dose (20 mg once daily) is outside the usual adult range for Sertraline (25 to 200 mg/day), so it may be a unit or entry error, or a medication error if it is correct.

Final classification. Total Naranjo score is 6, which is Probable (5 to 8). If a real alternative cause were confirmed later (Q5 = Yes, -1), the score would drop to 5, which would still be Probable. To firm this up I would want details of the restart, the medical history and any objective test results.